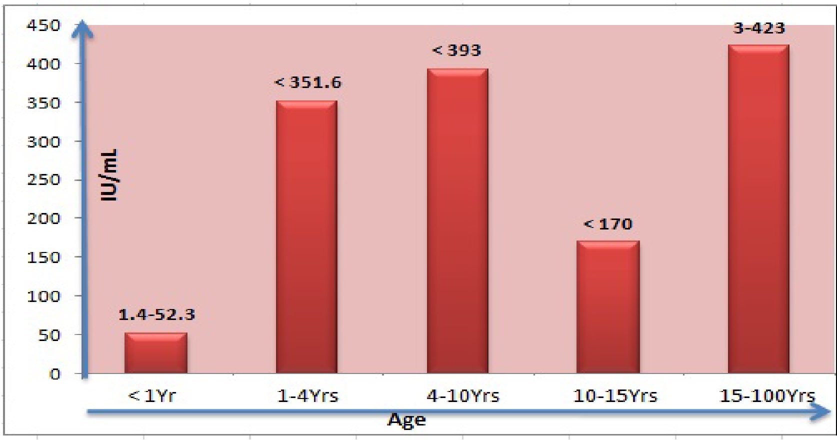
MISS SRIJA
Age : 15 Year(s)
Gender : Female

APL Code :APL-AP-1992
Ref Doctor :
Ref Cust :LIFE LINE DIAGNOSTICS

Sample Type : SERUM
SID : **12526226**
Collected on : 2026-04-15 00:00
Regd on : 2026-04-15 11:13
Reported on : 2026-04-15 00:00

CLINICAL BIOCHEMISTRY

Test Description	Result	Units	Biological Reference Ranges
Total IgE *(Method: LETIA)*	16.88	IU/mL	< 1Year:1.4 - 52.3 1 - 4 Years:0 - 351.6 4 - 10 Years:0 - 393.0 10 - 15 Years:0 - 170.0 15 - 100 Years : 3 - 423.0

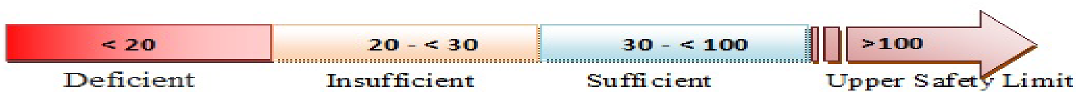


An elevated concentration of total immunoglobulin E (IgE) is not diagnostic for allergic disease, and must be interpreted in the clinical context of the patient, including age, gender, travel history, potential allergen exposure, and family history.A normal concentration of total IgE does not eliminate the possibility of allergic disease. In patients with a high index of suspicion for allergic disease, testing for allergen-specific IgE is recommended

Test Description	Result	Units	Biological Reference Ranges
Calcium *(Method: Spectrophotometry(Cresol Complex))*	9.3	mg/dL	8.8 - 10.6
25-Hydroxy Vitamin D Total (D2 & D3) *(Method: Chemiluminescence)*	**17.98**	ng/mL	Deficient : <20 Insufficient : 20 to <30 Sufficient : 30-100 Upper Safety Limit : >100

Interpretation:

Vitamin D levels in ng/mL



Vitamin D has several important functions. Perhaps the most vital are regulating the absorption of calcium and phosphorus, and facilitating normal immune system function. Getting a sufficient amount of vitamin D is important for normal growth and development of bones and teeth, as well as improved resistance against certain diseases. If body doesn't get enough vitamin D, risk of developing bone abnormalities such as soft bones (osteomalacia) or fragile bones (osteoporosis).Few foods contain vitamin D naturally. Foods that contain vitamin D include:salmon,sardines,egg yolk, shrimp, milk (fortified),cereal (fortified),yogurt (fortified),orange juice (fortified).It can be hard to get enough vitamin D each day through sun exposure and food alone, so taking vitamin D supplements can help.

Dr M.J.ANIL
Biochemist

Dr. SAI ABHISHEK VELPURI
MD PATHOLOGIST

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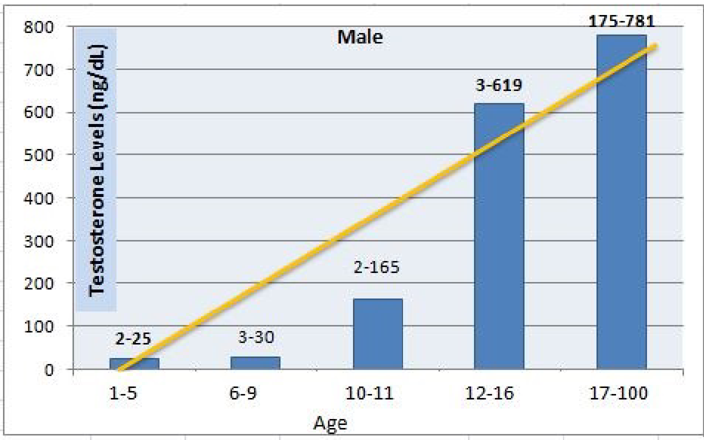
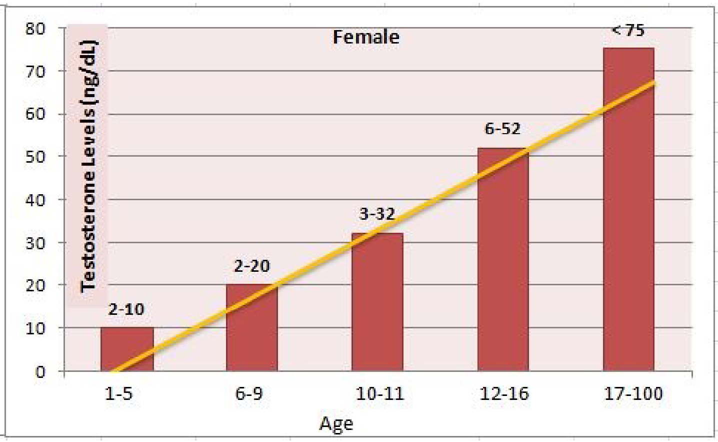
CLINICAL BIOCHEMISTRY

Test Description	Result	Units	Biological Reference Ranges
Testosterone - Total *(Method: Chemiluminescence)*	22.34	ng/dL	6 - 52





Test Description	Result	Units	Biological Reference Ranges
Vitamin - B12 *(Method: Chemiluminescence)*	371	Pg/mL	Deficiency : < 145 Indeterminate : 145 – 180 Normal : 180 - 914

Interpretation :

Deficient levels of vitamin B12 may cause megaloblastic anemia and peripheral neuropathies. Follow-up with a test for antibodies to intrinsic factor is recommended to identify this potential cause of vitamin B12 malabsorption. For specimens without antibodies and the patient is symptomatic, follow-up testing for vitamin B12 tissue deficiency may be indicated. Plasma homocysteine measurement (HCYSP/Homocysteine,Total,Plasma) is a good screening test where a normal level effectively excludes vitamin B12 and folate deficiency in an asymptomatic patient. However, the test is not specific and many situations can cause an increased level.

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CLINICAL BIOCHEMISTRY

Test Description	Result	Units	Biological Reference Ranges
LIPID PROFILE			
Cholesterol - Total *(Method: CHOD/PAP)*	140	mg/dL	< 200 : Desirable 200-239 : Borderline risk > 240 : High risk
Cholesterol - HDL *(Method: Direct)*	45	mg/dL	< 40 : Low 40 - 60 : Optimal > 60 : Desirable
Cholesterol - LDL *(Method: Homogeneous enzymatic end point assay)*	71	mg/dL	< 100 : Normal 100 - 129 : Desirable 130 – 159 : Borderline-High 160 – 189 : High > 190 : Very High
Cholesterol VLDL *(Method: Calculation)*	24.2	mg/dL	7 - 40
Triglycerides *(Method: Lipase / Glycerol Kinase)*	121	mg/dL	< 150 : Normal 150–199 : Borderline-High 200–499 : High > 500 : Very High
Total cholesterol/HDL ratio *(Method: Calculation)*	3.1	Ratio	0 - 5.0
LDL / HDL Ratio *(Method: Calculation)*	1.6	Ratio	0 - 3.5

Interpretation:

Lipid profile can measure the amount of Total cholesterol's and triglycerides in blood:

Test	Comment
Total cholesterol :	measures all the cholesterol in all the lipoprotein particles
High-density lipoprotein cholesterol (HDL-C):	measures the cholesterol in HDL particles; often called "good cholesterol" because HDL-C takes up excess cholesterol and carries it to the liver for removal.
Low-density lipoprotein cholesterol (LDL-C):	measures the cholesterol in LDL particles; often called "bad cholesterol" because it deposits excess cholesterol in walls of blood vessels, which can contribute to atherosclerosis
Triglycerides :	measures all the triglycerides in all the lipoprotein particles; most is in the very low-density lipoproteins (VLDL).

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CLINICAL BIOCHEMISTRY

Test Description	Result	Units	Biological Reference Ranges
IRON PROFILE			
Iron *(Method: Ferene)*	**15.4**	µg/dL	37 - 165
Iron Binding Capacity - Total (TIBC) *(Method: Ferrozine)*	421	µg/dL	240 - 450
Transferrin *(Method: Immunoturbidometry)*	**286.4**	µg/dL	176 - 280
Transferrin % *(Method: Calculation)*	**3.7**	%	20 - 50

Interpretation:

Iron participates in a variety of vital processes in the body varying from cellular oxidative mechanisms to the transport and delivery of oxygen to body cells.

Serum iron concentration is decreased in many but not all patients with iron deficiency anemia; in acute or chronic inflammatory disorders such as acute infection, immunisation, and myocardial infarction; acute or recent haemorrhage; malignancy; kwashiorkor; late pregnancy; menstruation and nephrosis.

Iron levels may also be increased in acute hepatitis, lead poisoning, acute leukaemia, thalassemia or oral contraception. TIBC is decreased in chronic infections, malignancy, in iron poisoning, renal disease, nephrosis, kwashiorkor and thalassemia. Common causes for an increase in TIBC include iron deficiency anemia, late pregnancy, oral contraception and viral hepatitis.

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CLINICAL BIOCHEMISTRY

Test Description	Result	Units	Biological Reference Ranges
KIDNEY BASIC SCREEN			
Creatinine(Serum) *(Method: Enzymatic)*	0.6	mg/dL	0.26 - 0.77
Urea (Serum) *(Method: UV-Kinetic)*	29.1	mg/dL	17 - 43
Blood Urea Nitrogen (BUN) *(Method: Calculation)*	13.6	mg/dL	6.0 - 20.0
Blood Urea Nitrogen (BUN)/Creatinine *(Method: Calculation)*	**22.7**	ratio	6 - 22
Sodium(Serum) *(Method: ISE Direct)*	136	mmol/L	135 - 150
Potassium(Serum) *(Method: ISE Direct)*	4.2	mmol/L	3.5 - 5.0
Chloride(Serum) *(Method: ISE Direct)*	101	mmol/L	94 - 110
Uric Acid *(Method: Uricase)*	3.1	mg/dL	2.6 - 6.0

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CLINICAL BIOCHEMISTRY

Test Description	Result	Units	Biological Reference Ranges
LIVER FUNCTION PROFILE			
Bilirubin Total *(Method: Diazotised Sulphanilic Acid)*	0.3	mg/dL	0.3 - 1.2
Bilirubin Direct *(Method: Diazotised Sulphanilic Acid)*	0.1	mg/dL	< 0.2
Bilirubin Indirect *(Method: Calculation)*	0.2	mg/dL	0 - 1.0
Alkaline Phosphatase (ALP) *(Method: AMP Buffer)*	115	U/L	50 - 162
Alanine Transaminase (ALT/SGPT) *(Method: UV with pyridoxal - 5 - phosphate)*	33	U/L	< 35
Aspartate Aminotransferase(AST/SGOT) *(Method: UV with Pyridoxal-5-phosphate)*	27	U/L	< 35
Y- Glutamyl Transferase (GGT) *(Method: g-Glut-3-carboxy-4 nitro)*	16	U/L	4 - 24
Protein Total *(Method: BIURET)*	7.3	g/dL	5.7 - 8.0
Albumin *(Method: Bromocresol Purple)*	3.9	g/dL	3.5 - 5.2
Globulin *(Method: Calculated)*	3.4	g/dL	2.5 - 3.5
Albumin / Globulin Ratio *(Method: Calculation)*	1.1	ratio	1.0 - 2.1

Interpretation:

- Liver function test aid in the diagnosis of various pre hepatic,hepatic & post hepatic causes of dysfuntion in anemias,viral & alcoholic hepatitis and cholestasis of obstructive causes.
- The test encompasses hepatic excreatory,synthetic function and also hepatic parenchymal cell damage.
- LFT helps in evaluating severity,monitoring therapy and assessing prognosis of liver disease and dysfunction.

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CLINICAL BIOCHEMISTRY

Test Description	Result	Units	Biological Reference Ranges
THYROID PANEL I			
TriIodothyronine Total (TT3) *(Method: Chemiluminescence)*	125.69	ng/dL	126 – 258: 1 Yr – 5 Yr 96 – 227 : 6 Yr – 15 Yr 91 – 164 : 16 Yr – 18 Yr 87 – 178 : > 18 years Pregnancy : 1st Trimester : 81 - 190 2nd & 3rd Trimester:100 - 260
Thyroxine - Total (TT4) *(Method: Chemiluminescence)*	9.41	µg/dL	6.09 - 12.23 Pregnancy: 4.6-16.5 : 1st Trimester 4.6-18.5 : 2nd & 3rd Tri
Thyroid Stimulating Hormone (TSH) *(Method: Ultra-sensitive chemiluminescence)*	2.47	µIU/mL	0.46 - 8.10 : 1 yrs-5 Years 0.36 - 5.80 : 6 yrs-18 Years 0.38 - 5.33 : 18 yrs-88 Years 0.50 - 8.90 : >88 Years Pregnancy ranges: 1st Trimester:0.05 - 3.70 2nd Trimester:0.31 - 4.35 3rd Trimister:0.41 - 5.18.

Clinical Features of Thyroid disease

Hypothyroidism	Hyperthyroidism	Grave's disease
• Lethargy • Weight gain • Cold intolerance • Constipation • Hair loss • Dry skin • Depression • Bradycardia • Memory impairment • Menorrhagia	• Tachycardia • Palpitations • Hyperactivity • Weight loss with increased appetite • Heat intolerance • Sweating | Diarrhea | Fine tremor | Hyper reflexia | Goitre • Palmar erythema • Onycholysis • Musle weakness and wasting • Oligomenorrhea / amenorrhoea	• Exophthalmos/proptosis • Chemosis • Diffuse symmetrical goiter • Pretibial myxoedema • Other autoimmune conditions • Thyroid bruit

Dr M.J.ANIL
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MISS SRIJA	APL Code :APL-AP-1992	Sample Type : WB EDTA
Age : 15 Year(s)	Ref Doctor :	SID : **12526225**
Gender : Female	Ref Cust :LIFE LINE DIAGNOSTICS	Collected on : 2026-04-15 00:00
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CLINICAL BIOCHEMISTRY

Test Description	Result	Units	Biological Reference Ranges
GLYCOSYLATED HEMOGLOBIN (HbA1c) *(Method: ion-exchange high-performance liquid chromatography(HPLC))*			
Hemoglobin A1c (HbA1c)	5.5	%	< 6 : Non Diabetic 6-7 : Good Control 7-8 : Poor Control > 8 : Alert
Estimated average glucose (eAG)	111.15	%	HbA1c(%) : eAG(mg/dL) 6 : 125 6.5 : 140 7 : 154 7.5 : 169 8 : 183 8.5 : 197 9 : 212 9.5 : 226 10 : 240

The A1c test is common blood test used to identify prediabetes ,diagnose type 1 and type 2 diabetes and to monitor how diabetes is managing .The A1c test result reflects your average blood glucose levels for the past two to three months ***American Diabetes Association*** recommends HbA1c monitoring frequency should be quarterly, particularly in case with suboptimal HbA1c conditions.

HbA1c(%)	eAG(mg/dl)	Condition	Severity
6.0	125	Non Diabetic : < 6.0	
6.5	140	Good Control : 6.0 - < 7.0	
7.0	154		
7.5	169	Poor Control : 7.0 - < 8.0	
8.0	183		
8.5	197	Diabetic > 8.0	
9.0	212		
9.5	226		
10.0	240		

Dr M.J.ANIL Biochemist		Dr. SAI ABHISHEK VELPURI MD PATHOLOGIST

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HAEMATOLOGY

Test Description	Result	Units	Biological Reference Ranges
Erythrocyte Sedimentation Rate (ESR) *(Method: Westergren's method)*	**25**	mm/Hour	<12

K. Yadagiri
Group Leader

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HAEMATOLOGY

Test Description	Result	Units	Biological Reference Ranges
COMPLETE BLOOD PICTURE			
Hemoglobin *(Method: Spectrophotometry)*	**8.3**	g/dL	11.5 - 16.0
Erythrocyte Count (RBC Count) *(Method: Impedance)*	4.29	mil/µL	3.8 - 4.8
Packed Cell Volume(Hematocrit) *(Method: Calculated)*	**27.4**	%	40 - 47
Platelet Count *(Method: Impedance/ Microscopy)*	2.59	lakh/Cumm	1.50 - 4.50
Red Cell Indices *(Method: Automated 5 part Cell counter/ Calculated)*			
MCV	**64**	fl	83 - 101
MCH	**19.4**	pg	27 - 32
MCHC	**30.4**	g/dL	31.5 - 34.5
RDW-CV	**17.3**	%	11.5 - 14.5
Total Count and Differential Count *(Method: Impedance/Microscopy)*			
Total Leucocyte Count(WBC)	**12900**	cells/Cumm	4000 - 11000
Neutrophils	40	%	40 - 75
Lymphocytes	**50**	%	20 - 40
Eosinophils	02	%	0 - 6
Monocytes	08	%	2 - 10
Basophils	00	%	0 - 1
Microscopic Blood Picture *(Method: Microscopy)*			
RBC Morphology	Anisocytosis, Microcytic Hypochromic Cells		
WBC Morphology	Leucocytosis with Lymphocytosis		
Platelet Morphology	Adequate		
Hemoparasites	Not found		
Impression	**Microcytic Hypochromic Anemia with Lymphocytic Leucocytosis**		
Advise	Correlate Clinically		

K. Yadagiri
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Dr. SAI ABHISHEK VELPURI
MD PATHOLOGIST